

Doc - Office-Clinic Notes



SANJAY GHOSH, MD, FAANS, FACS
Director of Neurological and Spinal Surgery

WWW.SANJAYGHOSHMD.COM

DOE: 11/16/2023

DOB: 12/7/1954

DOI: 10/18/2023

HISTORY: This is a follow-up visit on a very pleasant 68 year-old male who was evaluated by both myself and Dr. Ghosh. The patient was accompanied by his wife. The patient was involved in a MVA on 10/18/2023. The patient continues to have neck pain with difficulty turning his head. He has radiating pain into the left upper extremity. The patient is also having difficulty with fine motor functions of the hand. His handwriting has become illegible. His PCP has prescribed the patient gabapentin. He also has low back pain with radiating pain into the left lower extremity. He is here to review imaging studies.

PHYSICAL EXAMINATION:

Vitals: Blood Pressure: 136/87 Heart Rate: 87 Temperature: 98.8 Respiration: 16

Motor: 5/5 bilateral deltoids, biceps, triceps, grip. 5/5 bilateral knee extension/flexion, foot dorsi/plantarflexion, EHL

DIAGNOSTIC STUDIES:

MRI Cervical Spine: C6-7 disc herniation with moderate central, moderate severe left, and moderate right foraminal stenosis. C5-6 disc protrusion. C3-4 disc protrusion with moderate right and moderately severe left foraminal stenosis with cord signal changes consistent with myelomalacia.

Xrays Cervical Spine: C4-5 anterolisthesis with flexion and reduces upon extension.

MRI Lumbar Spine: L4-5, L5-S1 disc protrusions.

Xrays Lumbar Spine: No abnormal motion

ASSESSMENT:

Mr. Handrop is a 68 year-old male who was involved in a MVA on 10/18/2023. The patient has C6-7 disc herniation giving rise left C7 radiculopathy. He also has C5-6, C3-4 disc protrusions with myelomalacia of the cord at C3-4. He also has L4-5, L5-S1 disc protrusions.

PLAN:

1. Gentle physical therapy
2. Cervical ESI
3. RTC in 6 weeks or sooner with worsening symptoms

Felix Regala, MPAP, PA-C
Senta Neurosurgery

Supervising Physician: Sanjay Ghosh, MD